MANIPAL HOSPITALS - WHITEFIELD
Department of Pulmonology & Critical Care
Whitefield Main Road, Bangalore, Karnataka - 560066

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INPATIENT DISCHARGE SUMMARY

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PATIENT IDENTIFICATION:
Patient Name: Rajesh Kumar IP Number: IP-2026-04182
Age / Gender: 52 Years / Male UHID: MH-8821039
Admission Date: 2026-02-18 10:15 AM Discharge Date: 2026-02-21 02:00 PM
Consultant: Dr. Vikram Deshmukh, MD, DM (Pulmonology)

FINAL DIAGNOSES:
1. Acute Exacerbation of Bronchitis with Bronchospasm
2. Pre-existing Type 2 Diabetes Mellitus
3. Mild Systemic Hypertension

CHIEF COMPLAINTS AT ADMISSION:
- Productive cough with yellowish sputum for 5 days
- Progressive shortness of breath on exertion (mMRC Grade II)
- Low-grade fever (100.4°F) for 3 days
- Chest tightness and nocturnal wheezing

HOSPITAL COURSE & TREATMENT SUMMARY:
Patient presented with dyspnea and productive cough. Chest X-Ray on admission demonstrated bilateral peribronchial cuffing without focal consolidation or pleural effusion. He was treated with intravenous bronchodilators, short course IV Hydrocortisone, nebulization (Levosalbutamol + Ipratropium), and empirical antibiotic therapy (Azithromycin 500mg IV OD for 3 days). Glycemic levels were monitored and managed with regular insulin sliding scale during the acute steroid phase.
Patient responded well with clinical resolution of bronchospasm, clear lung fields, and normal SpO2 (98% on room air). Afebrile for 48 hours prior to discharge.

DISCHARGE MEDICATIONS:
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1. Tab. Cefuroxime Axetil 500 mg
   - Dosage: 1 tablet twice daily (BD) after food for 5 more days (Total: 7 days)
2. Inhaler Budesonide + Formoterol (200/6 mcg)
   - Route: Metered Dose Inhaler with Spacer
   - Dosage: 2 puffs twice daily for 30 days, then taper as per clinic review
3. Tab. Montelukast 10 mg + Levocetirizine 5 mg
   - Dosage: 1 tablet at bedtime (OD) for 14 days
4. Resume baseline anti-diabetic and anti-hypertensive regimen (Metformin & Lisinopril)

HOME CARE & WARNING SIGNS:
- Continue steam inhalation twice daily.
- Avoid exposure to dust, smoke, and cold air.
- Seek immediate medical attention at Emergency if experiencing:
  * Persistent high-grade fever (> 101°F)
  * Increased shortness of breath at rest or drop in SpO2 < 94%
  * Chest pain or hemoptysis (blood in sputum)

FOLLOW-UP APPOINTMENT:
Review with Dr. Vikram Deshmukh in Pulmonology OPD on February 28, 2026 at 11:00 AM.
Repeat Peak Expiratory Flow Rate (PEFR) and spirometry scheduled.

Prepared by: Dr. Rohit Verma, Resident Medical Officer
Approved by: Dr. Vikram Deshmukh, Senior Consultant Pulmonologist

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